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Examining the Inefficiency in the American Health Care System

Throughout the world, the American health care is known to be among the top in terms of its quality, research and use of modern technology. However, to American people, the reality may prove otherwise. In 2022, the US spends about 4.4 trillion dollars, or approximately 17.3 percent of its entire GDP on health care alone, which is huge compared to other OECD countries (Statista, 2023). Despite this, over 70 percent of American people believe their health care is failing them (Ducharme, 2023). Under this capitalist system in which private insurances are widely relied upon, one would expect these companies to take care for the majority of the financial burdens. Yet, the average American people still worry about the absurdly high prices of their hospital bills and pharmaceutical drugs since the insurances do not fully cover the costs that they are spending. More than half of Americans think the costs are putting a strain on their financial stability, which explains why there are many people who would rather stay at home and avoid visiting their doctors altogether (Ducharme, 2023). What actually lies underneath this system of inefficiency is the intricate network of hospitals, private insurance companies, drug manufacturers and pharmacy benefit managers who work together in the bid for achieving the highest profit. Through lobbying and passing various legislative bills, these pharmaceutical firms are able to obtain much influence in shaping American politics and society for better or worse. The consolidation of hospitals, or rather the private monopolization of the health care sector, and gouging of medical prices have all led to these increased health expenses, limited accessibility and mediocre health care quality for millions of Americans. This paper aims to explain why America ends up like this by looking deep into the structure of the US health care

system, such as the process of merging hospitals, and the drug pricing policies, as well as the problems and consequences surrounding them.

The US has one of the most complex health care systems in the world. It functions as a mixed system where both governmental and private entities act as intermediaries when it comes to distributing the health care good and services, as well as finances, between the American people who need treatments and the health care providers. Paying for health care come in two ways: the first one is public in which individual households pay for their health care via taxes to the state or federal government (using programs like Medicaid and Medicare); the second method is private in which people pay through their private health insurances in terms of monthly premiums—in some cases, their employers may pay their health insurances for them (ISPOR, 2024). All of this money would then go to the health care providers. On top of this convoluted nature of the financing system, what pushes hospital bills to be as high as they are for the American people despite having insurances and/or social programs to aid them?

Over the recent decades, America has experienced a persistent, yet almost exponential, increase in hospital care expenses. While inflation can certainly play a factor, this phenomenon is primarily attributed to the consolidation of the hospital system, where hospitals and other medical firms would merge with one another for a variety of reasons—some of which include the desire to reduce costs through shared resources and to maintain their market competitiveness. It started with the Health Maintenance Organization (HMO) Act of 1973, which aims to promote the growth of managed health care organizations, but this has paved the way for the privatization of the health care industry (Ballotpedia). Later in 2010, the Patient Protection and Affordable Care Act (or Obamacare) was passed to expand health care coverage, but has also incentivized hospitals to merge with one another by forcing people to purchase comprehensive insurance, requiring insurance companies to have standardized benefit packages and increasing subsidies to protect larger hospitals (Pope, 2014).

There are two types of consolidation. The first one is vertical, meaning a firm that produces one part of a product merges with another that makes another part of the product to own or control the entire supply chain. The second type is horizontal in which firms in the same industry that produce a similar product would merge with one another to reduce competition. With regards to hospitals, horizontally integrated structures include physicians with unique specialties, either individually or in groups, and multihospital systems that provide similar health care services in multiple places; on the other hand, vertically integrated structures include physician-hospital organizations (PHOs), management services organizations (MSO) and integrated delivery systems in which all aspects of distinct hospital services are combined into one (Heeringa et al., 2020). However, the integration of hospital system shifted more towards the vertical approach between the 1980s and 1990s (Heeringa et al., 2020). This trend has gained prominence especially during the COVID-19 pandemic in 2021 as about 75 percent of hospitals and 50 percent of all physicians throughout the 635 identified health systems are associated with vertical integration (Contreary et al., 2023).

This consolidation of hospital has come with its share of controversies. The merging of hospitals can possibly increase efficiency and quality of care thanks to the closer collaboration between different sectors of this new system, which in turn could reduce the overall costs and maximize the profits that the firm owner could gain. However, from an ethical perspective, the process has led to an unnecessary increase in costs for consumers as there would be even less competition in the market and less accessibility to such health care services. In one study which compares 246 hospitals that had undergone the process of consolidation to 1,986 other hospitals which have not, it is found that the treatment group “was associated with modestly worse patient experiences and no significant changes in readmission or mortality rates” (Beaulieu et al., 2020). In essence, the merging of hospitals does not correlate with any change in terms of quality of service, meaning that not only is there no actual benefit for the consumers,

they also lose more by having to pay for more and receiving less quality care. As for the firms owning these hospitals, they have likely succeeded in gaining more profits.

Furthermore, the consolidation of hospitals has exacerbated the lack of access to healthcare for communities that are vulnerable and isolated. Based on a 2023 data by the Center for Healthcare Quality and Payment Reform, “over 150 rural hospitals had closed between 2005 and 2019 [while] an additional 18 rural hospitals closed in 2020, more than any year in the previous decade” (CHQPR, 2023). Currently, almost a third of all hospitals in the rural areas, or about 600 of them, are at risk of closure due to lacking financial assistance (CHQPR, 2023). In another study from 2023, it is said that about 7 percent of unprofitable hospitals actually closed, while 17 percent decided to merge with larger hospitals firms outside their area (Carroll et al., 2023, p. 498). These statistics give a glimpse of the dire situation that affected communities now have to face. Without an easy access to healthcare, either people living in these areas would then have to travel a longer way to reach any health care centers that are in the cities, thereby increasing their expenses, or they would not make any attempt to seek help at all. Instead of opening hospitals in every area to increase health care accessibility, under this health care system, every hospital is used for profit only, with patients being treated as customers.

Meanwhile, pharmaceutical drug prices have also skyrocketed. Between 2007 to 2018, the “list prices increased by 159%, or 9.1% per year, while net prices increased by 60%, or 4.5% per year” (Hernandez et al., 2020, p. 854). As early as 2024, it is expected that more than 500 different drugs will have their prices increased as well (Erman and Wingrove, 2023). It should be noted that even without this jump in prices, pharmaceutical drugs in the US are comparatively very expensive compared to those in other OECD countries despite sharing the same brand and quality. For example, in 2017, one pill of Benicar Olmetec (20mg) is about \$5.07 in the US, whereas in Canada and the UK, it is only about \$1.37 and \$0.82 respectively (Miller, 2018). Furthermore, as of 2014, it is estimated that the average American spends up to \$1,112 per

person each year, while in Canada, each individual only spends \$772 each year (Miller, 2018). Moreover, a research study from 2017 finds that “70% of top-selling brand name drugs sold in US pharmacies are manufactured outside the country” and that “these same drugs are sold for up to 87% less in Canada and 97% less in other countries” (PharmacyChecker, 2017). These numbers really show the excessively high prices that Americans are forced to pay. Some people would rather travel to other countries, like Canada and Mexico, to buy drugs from there as they are cheaper. Most who do so tend to be senior citizens, Hispanic, have lower family income and lack of insurance (Hong et al., 2020). The factors that contribute to this situation is very similar to those that caused hospital bills to be as high as they are.

In 1980, the US government passed the Bayh-Dole or Patent and Trademark Law Amendments Act, which allowed businesses, universities and other non-profit organizations that receive federal funding to have ownership of any invention or research that they gain from that funding (University of Wisconsin-Madison, 2024). In 2003, the Medicare Modernization Act was signed into law by President George W. Bush, which introduced major changes to the Medicare program by creating the Medicare Part D prescription drug benefit to expand the coverage to people, especially senior citizens. However, like many other bills mentioned earlier, this also comes with a caveat in the form of the non-interference clause—it prohibits the Secretary of Health and Human Services from negotiating drug prices directly with pharmaceutical companies (Reed, 2023). This new policy effectively strengthens the drug industry monopolies as there would be no government agency to regulate the pricing of their products.

Pharmaceutical benefit managers (PBMs) are companies that play a crucial role in determining the price of drugs. They act as the intermediaries that negotiate such prices between various stakeholders in the health care system, like drug manufacturers and insurance companies. PBMs are initially created to help lower the drug prices, but “as PBMs demand larger and larger rebates or discounts, manufacturers offset these reductions by raising the “list” price for their drugs [in which] PBMs encourage this practice because they receive larger

financial rebates from more expensive drugs” (Gale, 2023). Additionally, the largest PBMs, such as CVS, Caremark, Express Scripts and Optum Rex control about 80 percent of the pharmaceutical drug market (Gale, 2023). It is estimated that about two-thirds, or 69 percent, of all people with private health insurance are being covered by the PBMs themselves—the majority of health insurance companies are vertically consolidated into the PBMs (Morse, 2022). Together with the restrictions outlined by the Medicare Modernization Act of 2003, these PBMs have essentially almost absolute control over how much they can set the prices for the drugs. Some may argue that the nature of the capitalistic market could lower the costs via competition, but in practice, this is not the case, as mentioned earlier.

The outcome of the aforementioned policies has far-reaching consequences on the health of the nation. Many Americans, especially those from marginalized and disadvantaged communities, are heavily affected by the staggering costs of health care. People living in poverty more than likely do not have adequate finance to pay for any insurances and so their access to health care services would be very limited. They are also likely to lack access to good food and water, never mind medical prescriptions. A survey in 2021 finds that “Black individuals had an uninsured rate of 9.9%, Hispanic or Latino individuals had a rate of 17.7% and white individuals had a rate of 6%” (United Way NCA, 2023). The limited of health care availability in terms of location is worsened thanks to the consolidation of hospital systems. As such, people living in rural areas and other low-income areas would have a difficult time getting treatment at all, from having to deal with the cost of transportation and their ability to make appointments, go for regular checkups and much more.

The situation became notably more severe during the COVID-19 pandemic between 2020 and 2022. For example, about “15,571 COVID-19 patients died at these hospitals due to lack of ICU beds during the month of April [in 2020]” (Belli, 2021). Additionally, it is found that “for every additional ICU bed per COVID-19 case, there was an associated one-fifth decrease in incidence of death” (Belli, 2021). The pandemic has proven to be a key moment when the

American health care system is as inefficient as it could be. The result is that minority communities have been hit the hardest. When adjusting for age distribution differences, it is calculated that Native Americans, Hispanic, Pacific Islander and African Americans have the highest mortality rates, more than white and Asian Americans—for instance, there is about 614 deaths per 100,000 people for Native Americans compared to just 315 deaths per 100,000 people for white Americans (Gawthrop, 2023). While white Americans do have significantly higher death tolls out of any other race or ethnicity, their mortality rate proportional to their population is rather low. If only health care services cost less and are available in more areas, the death rate would be more equal for each group and most importantly, much less than the actual data.

To conclude, the American health care system is one that is deeply flawed and inefficient thanks to the various policies and legislatures that allowed private health care companies to monopolize the industry without any regulation. The consolidation of hospitals and insurance firms, as well as the PBMs, has all made the large health care companies to have more control in the market, ironically driving down competition in this capitalistic economy. Hospital care and pharmaceutical drug prices have surged dramatically in the past decades, while the average Americans can barely afford to live their lives paycheck-to-paycheck. Americans from poorer areas and communities of color are struggling even more due to the already existing socioeconomic injustice put in place by the system.

There are several solutions to fix this issue, such as changing the past laws to allow more governmental regulations, creating more transparency laws, adjusting drug prices to match their actual value at the very least, and adopting a single-payer health care system. These ideas are easier said than done, which is why it is extremely important for everyone to understand how the current American health care system works, so one can better voice the dissatisfaction and potentially be able to influence governmental bills via voting.

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